Mergers and Acquisitions and Firm Management: Evidence from Hospitals

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Introduction

Mergers and acquisitions reshape organizations

- M&A has large and wide-ranging consequences for firms, markets, and workers
  - Large literature on aggregate effects: financial outcomes [Kim and Singal, 1993, Andrade et al., 2001], productivity [Demirer and Karaduman, 2024], wages and employment [Gehrke et al., 2021]
  - In healthcare: M&A raises prices [Cooper et al., 2019, Gowrisankaran et al., 2015], mixed effects on quality [La Forgia and Bodner, 2024, Beaulieu et al., 2020, Eliason et al., 2020]

- A growing literature establishes that managers can impact firm performance
  - Managers influence financial outcomes [Bertrand and Schoar, 2003, Bandiera et al., 2020], productivity [Fenizia, 2022], and wages and employment [Minni, 2025, Frederiksen et al., 2020, Hoffman and Tadelis, 2021]
  - In healthcare: managers can affect quality [Bloom et al., 2020, Muñoz and Otero, 2025] and prices [Hankins et al., 2025]

- Yet less is known about the **impact of M&A on managerial and organizational restructuring** of acquired firms
  - Corporate finance literature on CEO turnover post-acquisition [Martin and McConnell, 1991, Jenter and Lewellen, 2015, Lehn and Zhao, 2006], but limited to one role

Two views on what happens to management after an acquisition

Retain existing management

- Preserves **firm-specific human capital**
- Knowledge of local networks, regulatory environment, community needs
- Organizations invest in incumbent managers over time

[Doeringer and Piore, 1971, Baker et al., 1994]

Replace or reduce management

- Implements acquirer's strategy and practices
- **Discipline**: corrects underperformance that made the firm an acquisition target
- **Integration**: installs loyal managers for coordination across the merged entity

[Manne, 1965, Jensen and Ruback, 1983, Martin and McConnell, 1991]

Predictions of managerial restructuring

- If acquirers **retain** existing management:
  - Low turnover or disruption to management
- If acquirers **replace or reduce** management:
  - *Discipline*: targeted replacement of underperformers; more turnover at poorly performing targets [Manne, 1965, Martin and McConnell, 1991]
  - *Integration*: broad restructuring to implement acquirer's strategy; turnover uncorrelated with pre-acquisition performance [Walsh, 1988]
- Variation in strategy by target firm characteristics, implications for value creation post-acquisition

This paper: changes in firm management in the US hospital sector

- **[Descriptive]** What does the market for hospital managers look like?
  - Facts on executive labor markets, career paths, and organizational structure across hospital ownership types
- **[Causal]** How does M&A reshape hospital management and organization?
  - Effects of acquisition on CEO turnover, C-suite composition, and management structure
- **[Exploratory]** How does managerial restructuring relate to downstream outcomes?
  - Association between management disruption and financial/clinical performance

Why hospitals?

- **Large and growing sector:** hospitals account for ~5.5% of US GDP, with direct spending over $1.3 trillion and 6.6 million employees (1 in 6 jobs!)
- **Extensive M&A activity:** hospital acquisitions accelerated post-ACA, reshaping market structure nationwide
- **Managers matter:** Growing evidence that managers can impact quality of care [Bloom et al., 2020, Muñoz and Otero, 2025, Hankins et al., 2025]
- **Institutional variation:** non-profit vs. for-profit ownership creates sharp differences in governance, objectives, and organizational structure
- **Research gap:** despite extensive work on M&A and hospital prices/quality, almost nothing is known about who manages these organizations or how M&A changes management

Preliminary findings

- Construct a matched employer-employee panel tracking the full management hierarchy across a near-universe of US hospitals (2009–2017)
- NFP and FP hospitals have distinct executive labor markets: NFPs promote internally with longer tenure; FPs hire externally with higher baseline turnover
- M&A triggers large, immediate increases in CEO and C-suite turnover
  - Majority of CEO turnover is joint (other C-suite depart simultaneously)
  - Turnover is uncorrelated with pre-acquisition target performance
  - More consistent with organizational integration than managerial discipline
- NFP hospitals drive the turnover effects; FP acquisitions restructure through role elimination

Setting and Data

Data sources

Data	Source	Key feature
Hospital managers	HIMSS Analytics Database (2009–2017)	Near-universe of US hospitals; standardized management roles + individual identifiers
M&A events	Updated Cooper et al. [2019]; AHA + FactSet, Irving Levin, SDC Platinum	Near-universe of general acute care hospital acquisitions
Hospital financials	Healthcare Cost Report Information System (HCRIS)	Revenue, costs, profit margins
Hospital quality	Hospital Compare	30-day mortality rates (AMI, HF, pneumonia)
Patient satisfaction	HCAHPS	Overall rating, willingness to recommend

Sample construction

- Start with HIMSS hospital entities observed 2009–2017
- Restrict to general acute care hospitals
- Require match to AHA annual survey
- ~4,500 general medical and surgical hospitals (including CAH) per year
  - ~60% non-profit, ~15% for-profit, ~25% government/public
- For difference-in-differences (DID): balanced panel around 5-year event window ($e \in \{-2, -1, 0, 1, 2\}$)
  - ~980 acquired hospitals, ~3,700 never-acquired controls

HIMSS data: tracking hospital managers over time

- Annual survey of hospital IT and management personnel
- Covers standardized roles: CEO, CFO, COO, CIO, department heads, clinical chiefs, IT directors
- Individual identifiers allow tracking people within and across hospitals over time
  - Substantial data cleaning effort: name standardization, fuzzy matching, role harmonization
- Validated against AHA annual survey for CEO identification

Role classification: C-suite and functional tiers

Category	Roles
Panel A: Core C-Suite	
Core C-Suite	CEO: Chief Executive Officer; CFO: Chief Financial Officer; COO: Chief Operating Officer; CIO: Chief Information Officer
Panel B: Functional Tier Classification	
Business Tier 1	CEO: Chief Executive Officer; CFO: Chief Financial Officer; COO: Chief Operating Officer
Business Tier 2	Business Office Head; Marketing Head; Purchasing Head; Patient Accounting/Revenue Cycle Head
Clinical Tier 1	Medical Staff Chief; Chief Nursing Head
Clinical Tier 2	Quality Head; OB Head; Cardiology Head; ER Director; OR Head; Ambulatory Care Head; Patient Safety Head; Pathology Chief; Laboratory Director; Pharmacy Head; Radiology Med Dir
IT/Legal/HR Tier 1	CIO: Chief Information Officer; Chief Compliance Officer; CSIO/IT Security Officer
IT/Legal/HR Tier 2	IT Director; HR Head; HIM Director; Facility Management Head; Director of Technology; Clinical Systems Director

Outcome measures

- Three role statuses observed in each hospital-year:
  - **Active**: role exists and is filled
  - **Vacant**: role exists but is unfilled
  - **Does Not Exist (DNE)**: role is not present at the hospital
- Turnover measure: identity of incumbent changed relative to prior year
  - Example: if hospital i has CEO Smith in 2013 and CEO Jones in 2014, that is a turnover event
  - Also captures: new hire into previously vacant/non-existing role; incumbent departs and role becomes vacant or eliminated

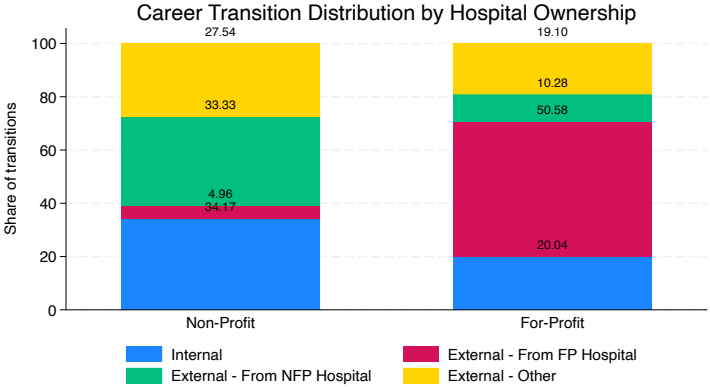
Descriptives: The Market for Hospital Executives

Hospital and CEO characteristics by ownership type, 2015

	All Hospitals	Non-Profit	For-Profit	Diff (NFP−FP)
Hospital characteristics				
Hospitals	4,528	2,627	667	
CEO present (%)	97.1	97.3	95.7	1.6**
COO present (%)	50.0	51.8	60.4	−8.6***
CFO present (%)	91.4	91.9	88.5	3.4***
Distinct roles (count)	24.2	25.1	21.9	3.1***
CEO characteristics				
Female (%)	24.8	25.3	22.1	3.2*
Clinical Degree (%)	14.9	16.4	10.7	5.7***
Years at Hospital (Any Role)	4.5	4.7	3.7	1.0***
Years as CEO (Any Hospital)	4.7	4.8	4.2	0.6***
Hospitals as CEO (Count)	1.5	1.5	1.5	0.0
Also CEO of Any System (%)	7.9	10.2	2.0	8.1***
Observations	4,395	2,556	638	

* $p < 0.10$, ** $p < 0.05$, *** $p < 0.01$. Difference is NFP minus FP.

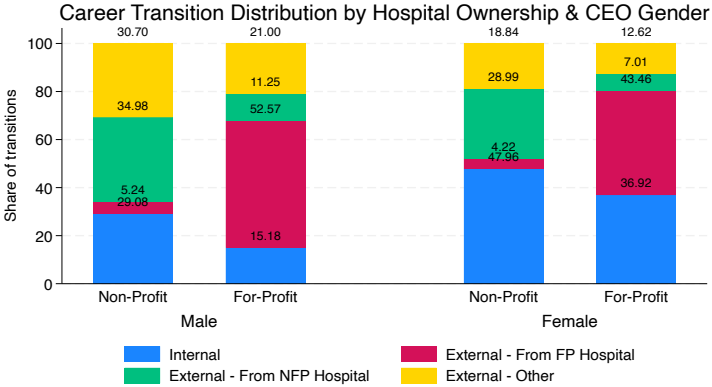
Non-profit hospitals rely on internal promotion; for-profits hire externally



Excludes External - Out of Sample transitions

Appendix: including out-of-sample transitions

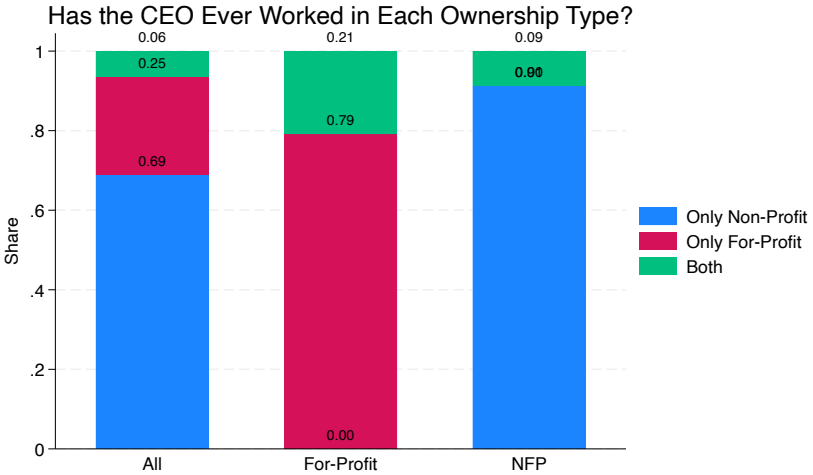
Women are more likely to be promoted internally in both ownership types



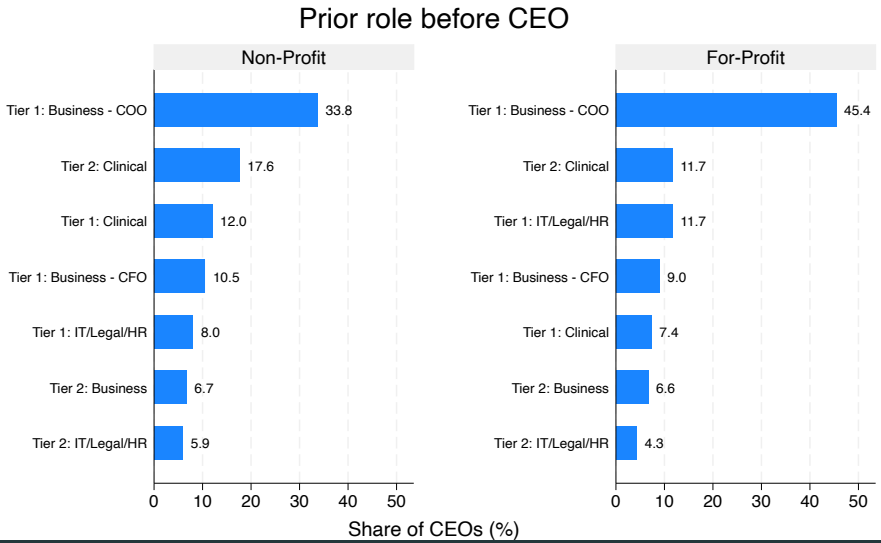
Excludes External - Out of Sample transitions

Appendix: including out-of-sample transitions

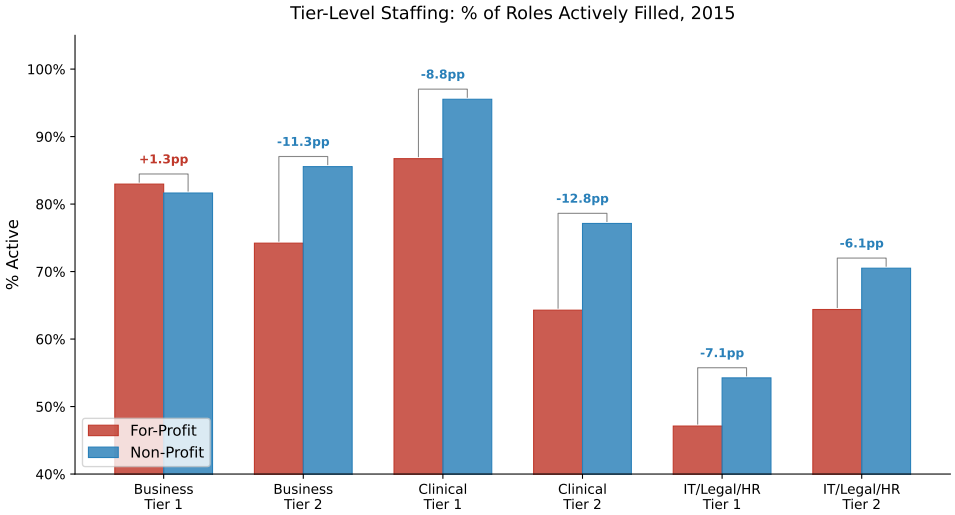
CEOs rarely cross between non-profit and for-profit sectors



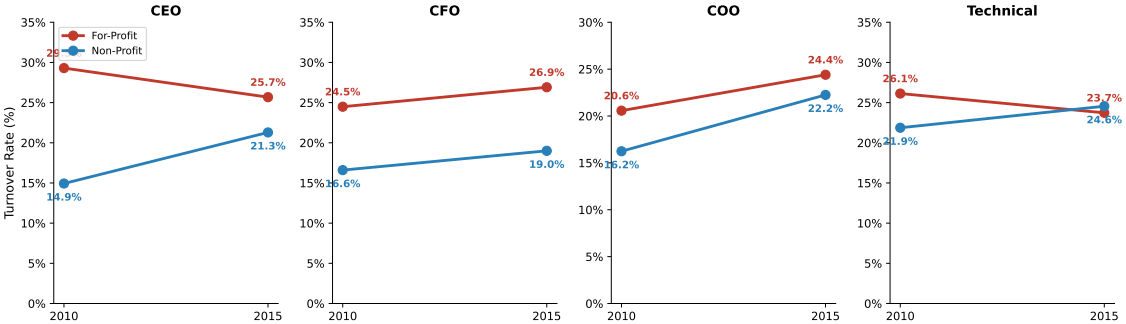
COO is the dominant path to CEO, but NFPs draw more from clinical roles



For-profit hospitals have leaner management structures



For-profit hospitals have higher executive turnover at baseline



Summary: two distinct labor markets and management structures

- NFP/FP governance differences are well-documented [Hansmann, 1980, Glaeser and Shleifer, 2001], but this is the first evidence on how these translate to differences in management structure and executive labor markets
- Non-profit hospitals rely more on internal labor markets for executives
  - Internal promotion, longer tenure, more fully staffed management hierarchies
  - COO is the primary path to CEO, but clinical roles are a substantial pipeline (~30%)
- For-profit hospitals rely more on external labor markets for executives
  - External hiring, shorter tenure, leaner management structures, higher baseline turnover
  - CEO pipeline heavily concentrated in COO (45%); clinical pathway is smaller
- Key implication: M&A may differentially disrupt these structures

How does M&A impact hospital management?

Research design

- DID event study design comparing management outcomes (e.g., executive turnover, role composition) in acquired hospitals (treated) vs. never-acquired hospitals (control)
- Identifying assumption: absent acquisition, management outcomes in treated and control hospitals would have followed parallel trends

Empirical strategy: event study around hospital acquisitions

$$Y_{it} = \alpha_i + \lambda_t + \sum_{\substack{e=-2 \\ e\neq -1}}^{2} \beta_e \cdot \underbrace{\mathbb{1}[\text{TimeToAcq}_{it} = e]}_{\text{event-time indicators}} + \varepsilon_{it}$$

- $e = -1$ is omitted reference period
- α_i: hospital fixed effects λ_t: year fixed effects
- Never-acquired hospitals serve as comparison group
- Standard errors clustered at the hospital level
- Interaction-weighted estimator to address staggered adoption bias [?]

Heterogeneity and decomposition

- Sub-group analyses by pre-acquisition hospital characteristics:
  1. Non-profit vs. for-profit target
  2. Outgoing CEO gender
  3. Other hospital characteristics Details

- CEO turnover decomposition:
  - *Outgoing*: solo CEO turnover vs. joint turnover (CEO + other C-suite)
  - *Incoming*: internal promotion vs. within-system hire vs. external hire

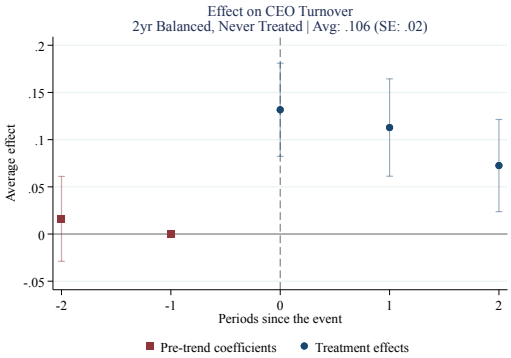
M&A sample summary statistics

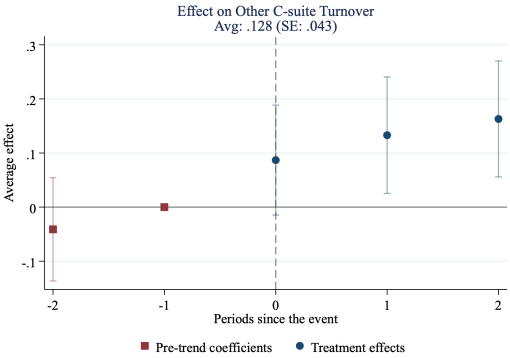
	Acquired (Pre)	Acquired (Post)	Non-Acquired
Ownership and structure			
For-profit (%)	19.95	25.70	12.41
Nonprofit (%)	68.66	66.76	60.40
Government (%)	11.39	7.55	27.20
Small system (%)	26.35	37.42	26.66
Large system (%)	32.85	56.89	29.35
Beds	199.17	184.71	167.25
CEO characteristics			
Female CEO (%)	18.23	25.40	21.93
MD CEO (%)	5.61	5.65	5.52
Financial and quality			
Profit margin (%)	2.68	2.37	4.03
AMI mortality (%)	15.35	14.36	14.97
HF mortality (%)	11.60	11.85	11.81
Top-box overall (%)	63.40	64.67	64.07
Top-box recommend (%)	64.73	64.61	64.39
Unique Hospitals	979	1,044	3,709
Hospital-Years	4,399	4,799	31,438

Results: What happens to hospital management after M&A?

CEO and Other C-Suite Results

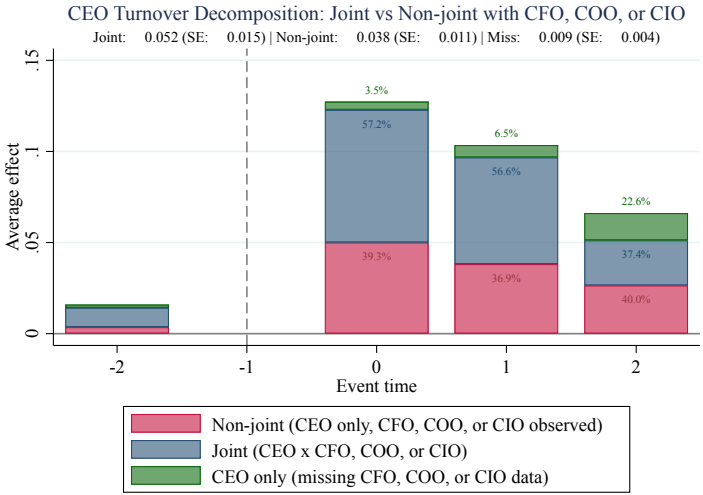
CEO turnover increases ~54% and other C-suite turnover ~23% post-acquisition



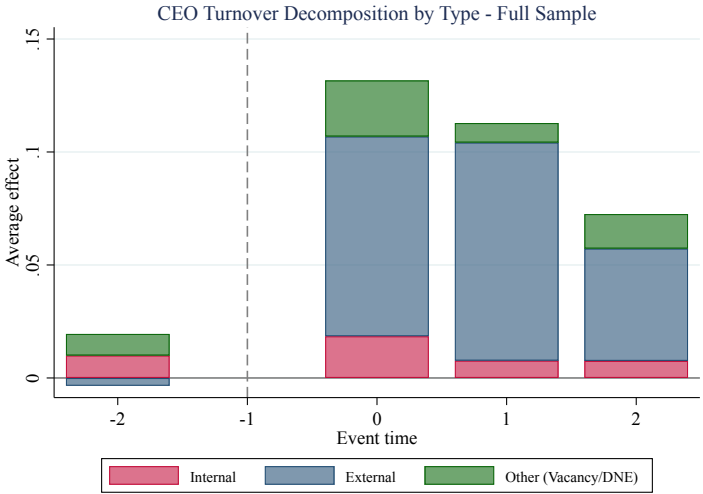


- Disruption extends beyond the CEO to CFO, COO, and CIO Forest plot by role

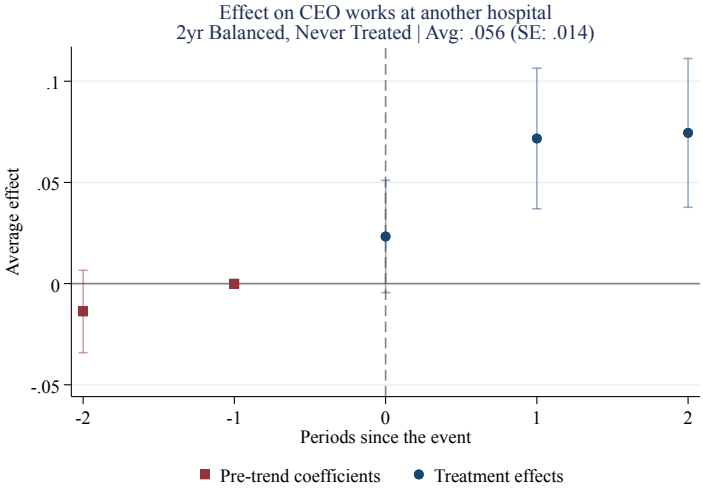
Post-acquisition CEO turnover is often joint with other C-suite members



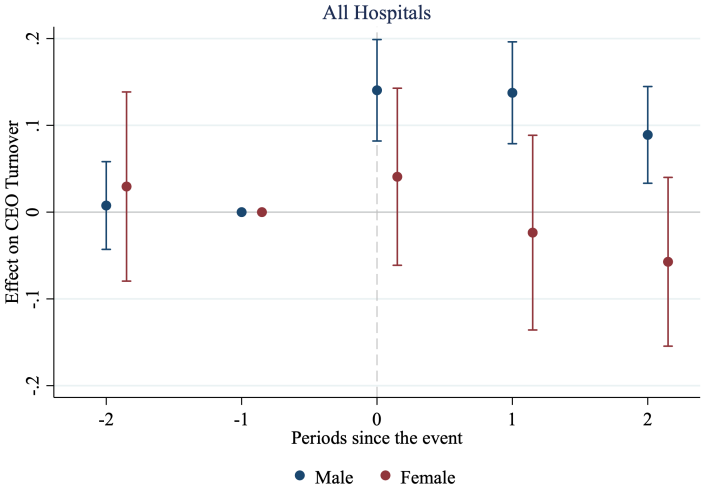
More incoming CEOs are external to the target hospital



Post-acquisition, CEOs are ~45% more likely to lead multiple hospitals



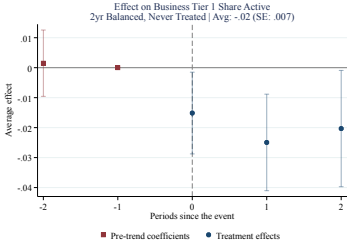
Post-acquisition CEO turnover is driven entirely by male CEOs



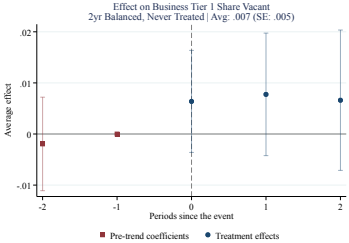
Restructuring of the Functional Tiers

Business Tier 1: active, vacant, and does-not-exist

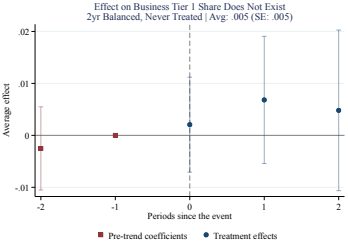
Roles: CEO, CFO, COO



Active



Vacant

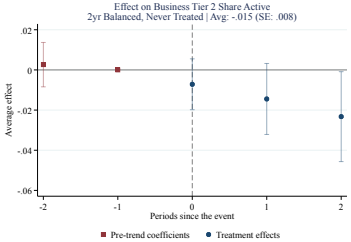


Does Not Exist

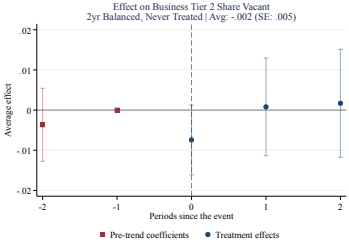
Decline in share active – combination of vacancy & DNE for CFO and COO

Business Tier 2: active, vacant, and does-not-exist

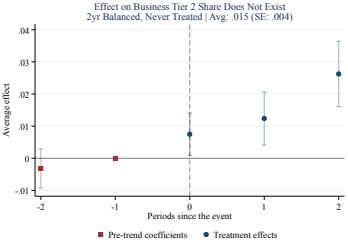
Roles: Business Office Head; Marketing; Purchasing; Revenue Cycle



Active



Vacant

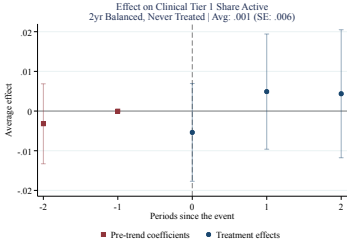


Does Not Exist

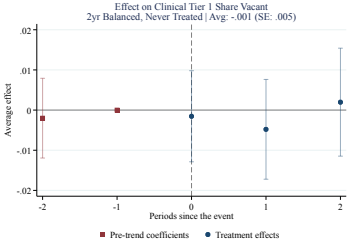
Decline in active roles driven by role elimination

Clinical Tier 1: active, vacant, and does-not-exist

Roles: Medical Staff Chief; Chief Nursing Head



Active



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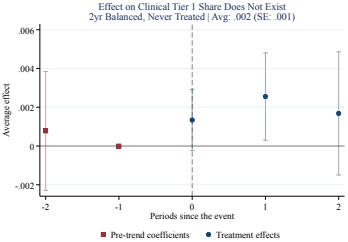
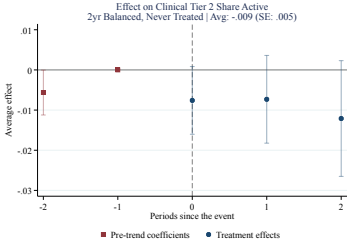


Does Not Exist

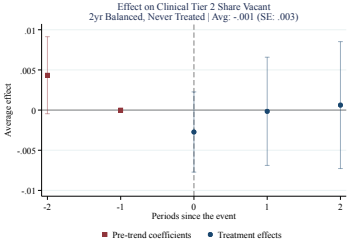
No meaningful changes in clinical tier 1

Clinical Tier 2: active, vacant, and does-not-exist

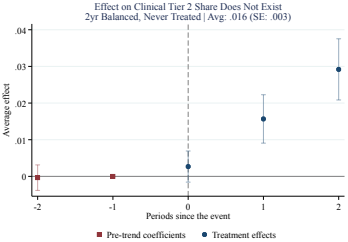
Roles: Quality; OB; Cardiology; ER; OR; Ambulatory Care; Patient Safety; Pathology; Lab; Pharmacy; Radiology



Active



Vacant

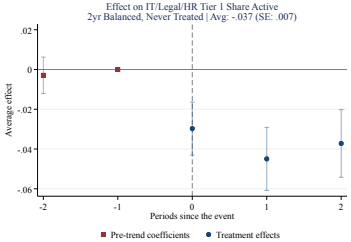


Does Not Exist

Significant and growing role elimination of clinical tier 2 roles

IT/Legal/HR Tier 1: active, vacant, and does-not-exist

Roles: CIO; Chief Compliance Officer; CSIO; Chief Medical Info Officer



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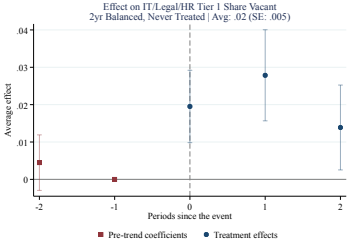
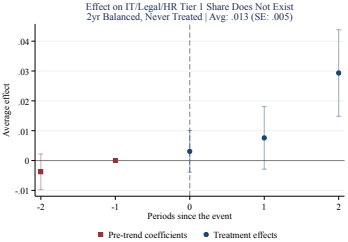


Vacant

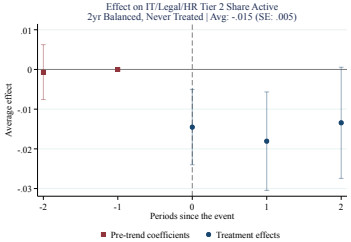


Does Not Exist

Large decline in share active driven by both increased role vacancies and elimination

IT/Legal/HR Tier 2: active, vacant, and does-not-exist

Roles: IT Director; HR; HIM; Facility Mgmt; Technology; Clinical Systems



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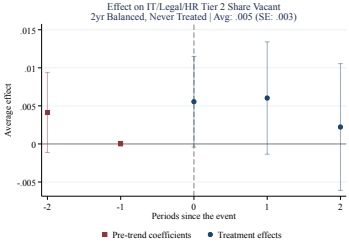
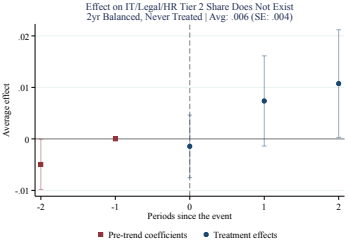


Vacant



Does Not Exist

Large decline in share active driven by both increased role vacancies and elimination

What Drives Managerial Restructuring?

Summary: M&A triggers broad managerial restructuring

- **C-suite turnover**: CEO turnover increases ~54%, other C-suite ~23% (more joint turnover); incoming CEOs are more external and more likely to lead multiple hospitals (+45%)
- **Tier-level restructuring**: M&A increases the share of roles that are vacant or altogether eliminated
  - Strongest in business and IT tiers, consistent with centralizing administrative and IT functions at the system level
  - Clinical tier effects are more muted; patient-facing roles may be harder to eliminate
- General pattern is more consistent with organizational integration rather than targeted discipline...

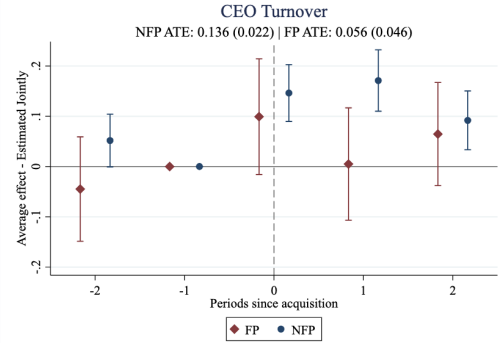
Little difference in effects by pre-acquistion hospital characteristics

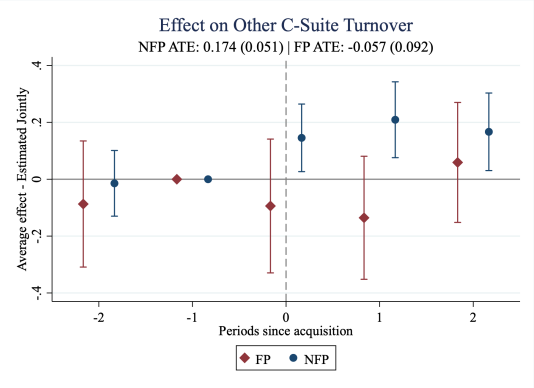
- Target hospital size, Medicaid/Medicare share, FTE, teaching status, acquirer size: not significantly associated with differential CEO turnover Appendix: heterogeneity by characteristic
- Pre-period hospital performance (clinical quality, patient satisfaction rating and hospital profit margin) has little association with turnover or restructuring
  - **Exception:** hospitals performing poorly on quality measures are more likely to replace their COO and CNO

  Appendix: quality heterogeneity
- Consistent with integration (systematic restructuring) rather than discipline (performance-targeted), with a possible narrow disciplinary channel for clinical leadership

Heterogeneity by Ownership Type

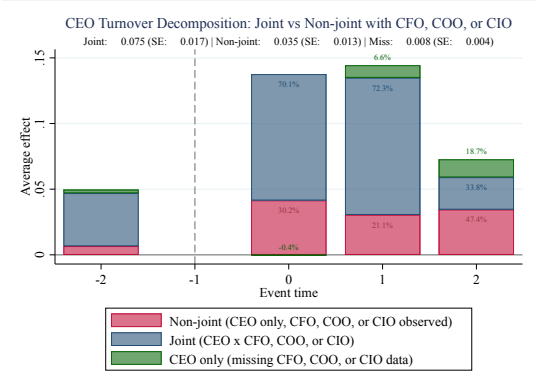
Non-profit hospitals drive the CEO and C-suite turnover effects



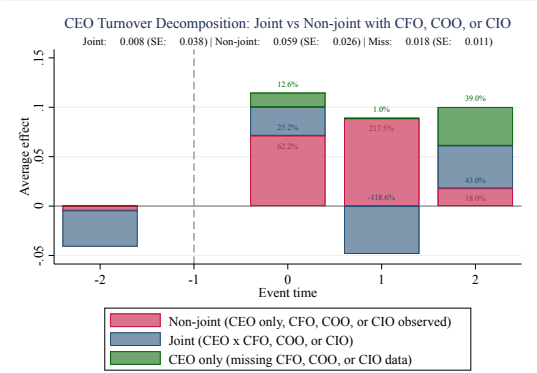


- NFP hospitals have higher M&A-induced turnover
- FP effects imprecisely estimated (smaller sample, wider CIs)

NFP CEO turnover is predominantly joint; FP turnover is mostly solo

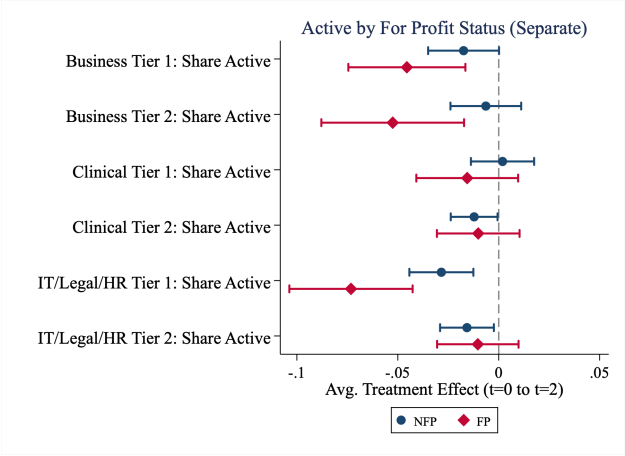


Non-Profit



For-Profit

For-profit acquisitions mostly drive the functional tier restructuring

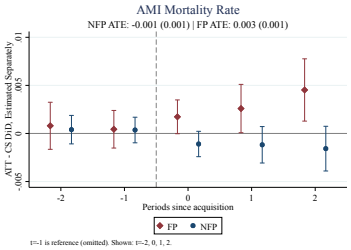


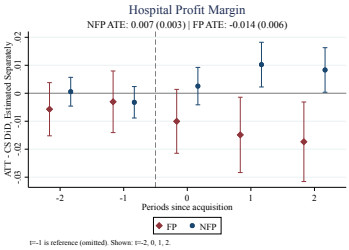
Next step: exploring differential drivers of NFP vs. FP restructuring

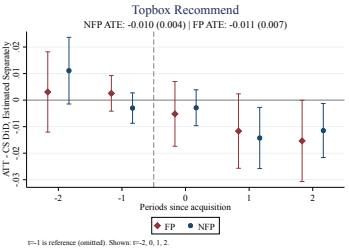
- NFP turnover appears broad-based and joint; FP turnover is more solo and targeted
- FP CEO turnover may reflect a disciplinary channel rather than systematic integration
- Need to examine how target hospital performance in the pre-acquisition period differs separately for NFP and FP targets
  - Does pre-acquisition clinical or financial performance predict CEO turnover differentially by ownership type?

Next Steps: Downstream outcomes

Direct effects of M&A on clinical and financial outcomes

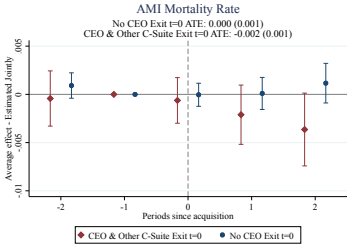




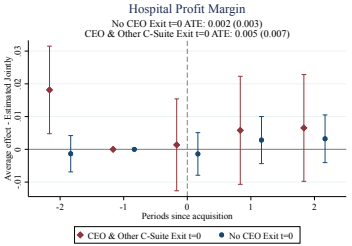


- NFPs had "turnover at the top" → some evidence of lower AMI mortality and higher profit margin
- FPs had less turnover but more role deletion/vacancy of functional tiers → some evidence of higher AMI mortality and lower profit margins
- Both hospital types had reduced patient satisfaction

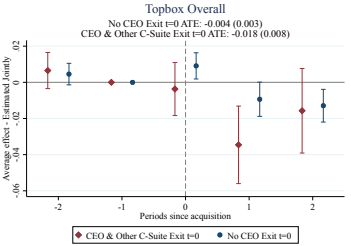
Is greater turnover associated with downstream outcomes?



AMI Mortality



Profit Margin



Patient Satisfaction

Discussion and Conclusion

Summary of findings

- **[Descriptive]** The hospital executive labor market is segmented by ownership type
  - NFPs promote more internally and FP hire more externally, and few CEOs work across these two types
  - First panel tracking the full management hierarchy across US hospitals
- **[Causal]** M&A triggers broad organizational restructuring, not targeted replacement
  - CEO turnover is predominantly joint with other C-suite departures
  - Turnover is uncorrelated with pre-acquisition target performance
  - NFP acquisitions drive turnover; FP acquisitions restructure through role elimination
- **[Exploratory]** Preliminary evidence that restructuring type is associated with downstream clinical and financial outcomes

Discussion

- Results favor **integration** over **discipline** view
  - Joint turnover, lack of performance-targeting, and broad restructuring are inconsistent with the discipline hypothesis [Manne, 1965, Martin and McConnell, 1991]
  - More consistent with acquirers installing managers for coordination across the merged entity [Walsh, 1988]
- NFP vs. FP acquirers use distinct restructuring strategies with potentially different consequences
  - Will continue to explore the NFP vs FP angle

Thank you!

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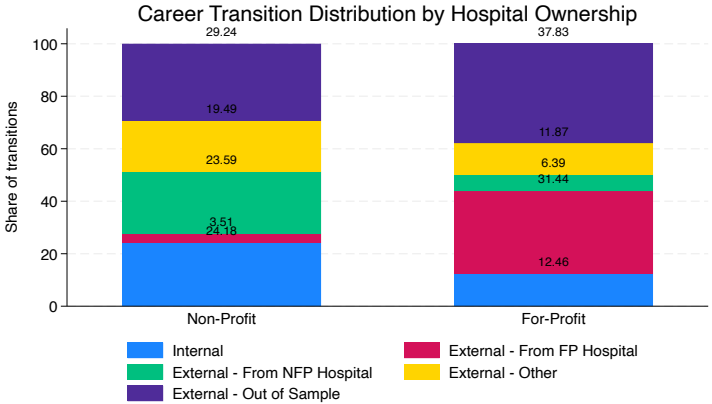
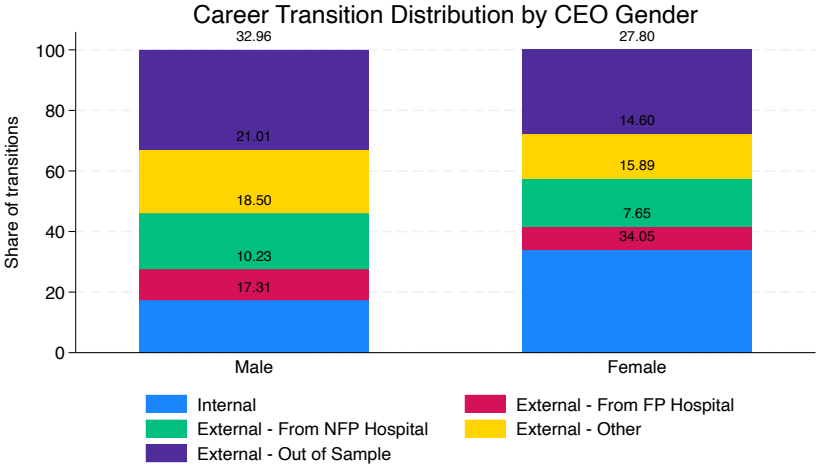
Appendix

CEO transitions by ownership type (including out-of-sample)

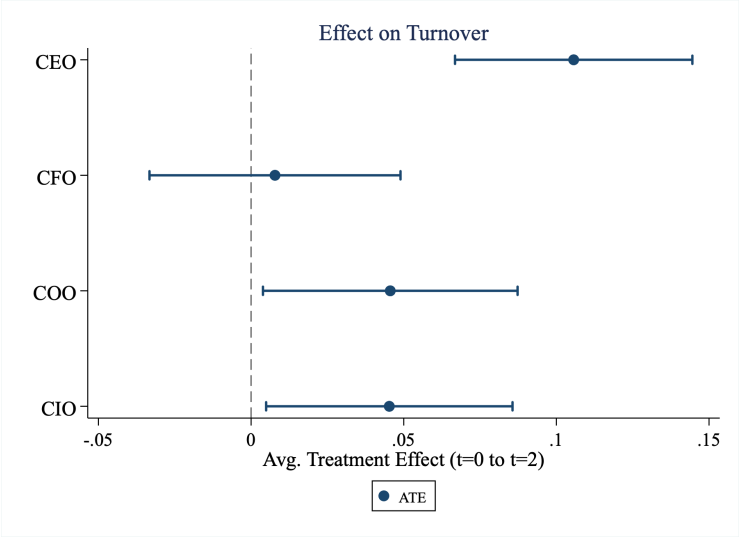


Back

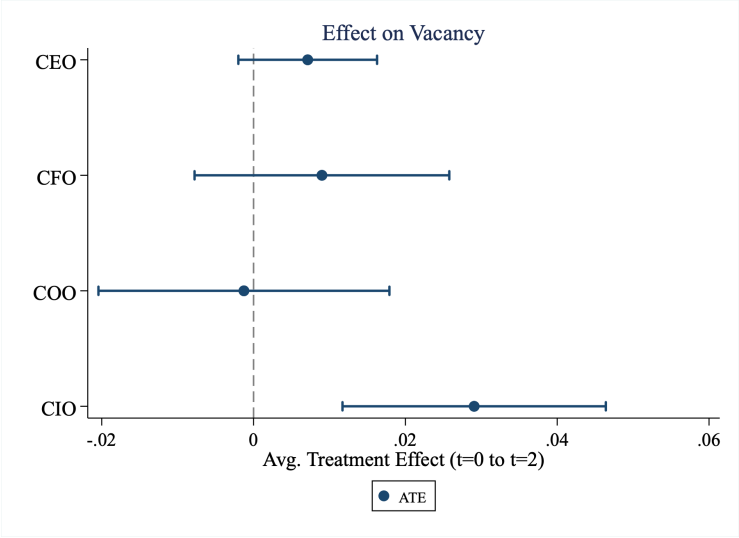
CEO transitions by ownership and gender (including out-of-sample)



Forest plot: C-suite turnover by role



Forest plot: C-suite vacancies by role



CEO turnover heterogeneity by hospital characteristics

figures/forest_turnover_raw_bdtot.png

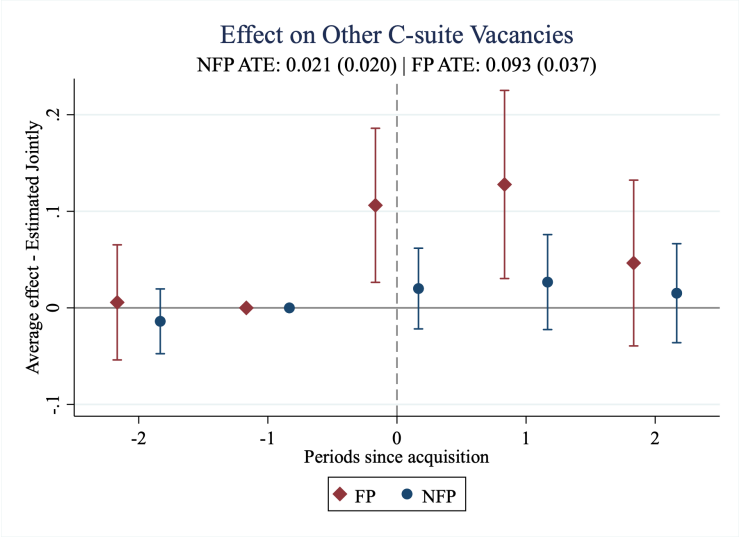
figures/forest_turnover_raw_teaching.png

Turnover heterogeneity by pre-period quality

figures/qhet_turnover_ami_mort_rate.png

figures/qhet_turnover_hf_mort_rate.png

C-suite heterogeneity by profit status: vacancies



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